

Concerns after the birth

Following the birth, it is natural to feel overwhelmed and wonder about the normal development of the baby or your recovery postpartum. The concerns in this section may require more attention from yourself or a health professional.

Maternal problems

Mastitis most commonly affects breastfeeding women. There may be localized redness, hardness, and soreness in the breast; the breast may feel swollen and hot; and there may be flulike symptoms.

Causes Mastitis is either non-infectious, caused by a blocked milk duct that allows milk to stagnate in the breast tissue, or infectious if bacteria get into the blocked duct. If untreated, it can develop into a painful abscess.

What to do Continue breastfeeding to help remove the blocked milk. Massaging the breast from under the arm while feeding can also help, as can expressing milk after a feeding. You should drink lots of fluids and apply warm compresses on the breast. Over-the-counter pain relievers such as acetaminophen can help. If an infection is present, antibiotics will be needed. An abscess must be surgically drained.

Bladder problems After giving birth, you may leak urine when you cough, sneeze, laugh, or move around, known as stress incontinence, or you may have a sudden, intense need to urinate referred to as urge incontinence or overactive bladder.

Causes Both conditions are caused by the stretching and weakening of pelvic-floor muscles and are exacerbated by excess fluid from pregnancy. Bladder problems should improve in the days and weeks after the birth as your pelvic muscles start to tone up.

What to do [Kegel exercises](#) to strengthen and tone muscles are recommended. If problems persist, your doctor may check that you're doing the exercises properly and that there are no other symptoms that could indicate an infection. Referral to a pelvic physical therapist or urologist may be beneficial.

Postpartum depression affects about 1 in 10 new mothers and usually develops 4–6 weeks after the birth but can come on any time in the first year. If left untreated, it can persist and have a serious effect on a woman's life. Symptoms include anxiety; irritability; tearfulness; panic; a prolonged low mood; an inability to cope; a lack of interest in appearance; difficulty in concentrating or getting motivated; and not bonding with the baby. Also feelings of inadequacy, guilt, rejection, and isolation; and not sleeping; fatigue; headaches; lack of appetite; loss of libido, stomach pains, and feeling unwell.

Causes Several factors may increase the risk of postpartum depression, such as a history of depression or mental health problems, a traumatic birth, or relationship problems.

What to do In mild cases, emotional and practical support may be sufficient. More serious cases are often treated with antidepressants; counseling or psychotherapy may be recommended.

Postpartum psychosis is a rare but serious psychotic illness that occurs in the first two weeks after the birth. The mother may be confused, unable to cope, may neglect her appearance, and forget to take care of her baby. In severe cases, she may have suicidal thoughts and could possibly harm her baby.

What to do This needs treatment from a psychiatrist. The mother may be admitted to a specialized hospital.